

Psychosocial Outcomes and Coping Mechanisms Among Women with Premenstrual Dysphoric Disorder: A Global Scoping Review

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Running head: PMDD PSYCHOSOCIAL OUTCOMES: A SCOPING REVIEW

Keywords: premenstrual dysphoric disorder; psychosocial outcomes; coping; suicidality; quality of life; low- and middle-income countries; sub-Saharan Africa; scoping review

Abstract

Background: Premenstrual dysphoric disorder (PMDD) is a DSM-5 depressive disorder with an estimated confirmed community prevalence of 1.6%, equivalent to roughly 31 million women and girls worldwide. PMDD is associated with depression, suicidality and impaired functioning, yet the global distribution of evidence on its psychosocial burden, and on how affected women cope, has not been mapped.

Methods: Following Arksey and O'Malley's framework and PRISMA-ScR, records exported from CINAHL Ultimate, APA PsycInfo, APA PsycArticles, MEDLINE Ultimate, Women's Studies International, PubMed and the Cochrane Library were screened against pre-specified criteria: peer-reviewed empirical studies (2010–2025, English) with PMDD as a defined population and at least one psychosocial outcome or coping mechanism as a primary aim. Animal, laboratory, intervention, review and instrument-validation studies were excluded. Data were charted for design, setting, World Bank income group, PMDD ascertainment and psychosocial domain.

Results: Of 1,801 records, 342 were duplicates; 1,459 were screened and 85 studies met inclusion criteria. Depression was the most frequently examined domain ($n = 28$), followed by mood and emotion regulation ($n = 18$), psychiatric comorbidity ($n = 18$), psychological distress ($n = 16$), suicidality and self-harm ($n = 13$), academic/occupational functioning ($n = 13$), interpersonal functioning ($n = 12$), quality of life ($n = 8$) and coping or resilience ($n = 7$). Only 20 studies (23.5%) confirmed PMDD with prospective daily ratings. Two-thirds of studies (57; 67.1%) came from high-income countries; 21 (24.7%) came from low- and middle-income countries (LMICs), but only 5 (5.9%) from low- or lower-middle-income countries and 3 (3.5%) from sub-Saharan Africa (Ethiopia, $n = 2$; Nigeria, $n = 1$).

Conclusions: Evidence on the psychosocial burden of PMDD is growing but remains concentrated in high-income settings, relies heavily on provisional diagnoses, and rarely examines coping or lived experience. Culturally responsive screening, integration of PMDD into sexual, reproductive and mental health services, and Africa-centred research investment are needed.

[Word count of abstract $\approx$ 330 words; trim to the target journal limit. Note that the counts above supersede those in the APHA abstract submitted before full screening—see the cover note.]

Introduction

Premenstrual dysphoric disorder (PMDD) is a cyclical, hormone-linked mood disorder in which marked affective symptoms—irritability, affective lability, depressed mood and anxiety—together with behavioural and physical symptoms emerge in the late luteal phase of the menstrual cycle and remit shortly after the onset of menses (American Psychiatric Association [APA], 2013). PMDD was placed in the depressive disorders chapter of the *Diagnostic and Statistical Manual of Mental Disorders* (5th ed.; DSM-5) in 2013 and is also recognised in the *International Classification of Diseases, 11th Revision* (ICD-11), which came into effect in 2022 (APA, 2013; World Health Organization [WHO], 2022). Its diagnostic validity nevertheless continues to be debated (Naik et al., 2023).

Estimates of how common PMDD is depend heavily on how it is diagnosed. A meta-analysis of 44 studies (50,659 participants) estimated a pooled point prevalence of 3.2% for confirmed diagnoses—which require prospective daily symptom ratings over at least two cycles—and 7.7% for provisional diagnoses based on retrospective report; restricted to community samples with confirmed diagnosis, prevalence was 1.6% (Reilly et al., 2024), equivalent to roughly 31 million women and girls worldwide (University of Oxford, 2024). Premenstrual symptoms more broadly are highly prevalent in Africa: a meta-analysis of 16 African studies reported a pooled premenstrual syndrome (PMS) prevalence of 47.0% (Andualem et al., 2024).

Because PMDD recurs every month across the reproductive years, its consequences extend well beyond symptom counts. Studies have linked PMDD with depression, suicidal ideation and behaviour, strained relationships, lost productivity and reduced quality of life, and have begun to examine how affected women cope. However, this evidence is dispersed across psychiatry, gynaecology, nursing and psychology journals, and it is unclear which psychosocial

outcomes have been studied, with what methods, and in which parts of the world. This matters for public health: in many low- and middle-income countries (LMICs), and particularly in sub-Saharan Africa, menstrual health remains under-prioritised and mental health services are scarce, so a condition whose burden is undocumented is unlikely to be recognised by health systems [add supporting citation, e.g., WHO Mental Health Atlas].

We therefore conducted a scoping review to map the global evidence on psychosocial outcomes and coping mechanisms among women with PMDD. The review addressed four questions: (1) Which psychosocial outcomes and coping mechanisms have been examined among women with PMDD? (2) How are these studies distributed across countries, regions and income settings? (3) What study designs and diagnostic approaches characterise this evidence? (4) What gaps exist, particularly for LMICs and sub-Saharan Africa?

Methods

Design

This scoping review followed the five-stage framework of Arksey and O'Malley (2005), with the methodological refinements proposed by Levac et al. (2010), and is reported according to the Preferred Reporting Items for Systematic Reviews and Meta-Analyses extension for Scoping Reviews (PRISMA-ScR; Tricco et al., 2018). Consistent with scoping-review methodology, the aim was to map the extent and nature of the evidence rather than to appraise study quality or pool effect estimates.

[State whether a protocol was registered (e.g., Open Science Framework) and give the registration DOI/date, or state that no protocol was registered.]

Information Sources and Search

Records were retrieved from CINAHL Ultimate, APA PsycInfo, APA PsycArticles, MEDLINE Ultimate and Women's Studies International (via EBSCOhost), PubMed and the

Cochrane Library. Search strategies combined terms for premenstrual dysphoric disorder (e.g., “premenstrual dysphoric disorder”, “PMDD”, “premenstrual disorder*”) with psychosocial and coping concepts. The exported records (1,801 in total) formed the screening set; the publication-date limit (January 2010 to December 2025) was applied at screening because some exports contained records outside this range.

[Insert the exact search string, limits and search date for each database in Appendix A. The PubMed export provided contributed 6 records; if the full PubMed search yielded more, add those records and re-run deduplication so that the PRISMA counts match.]

Eligibility Criteria

Eligibility was defined using the Population–Concept–Context framework. *Population:* women and people who menstruate with PMDD defined by DSM-IV, DSM-5 or ICD criteria, whether provisional (retrospective screening instruments such as the Premenstrual Symptoms Screening Tool) or confirmed (prospective daily ratings), including studies of premenstrual disorders that reported a distinct PMDD group or diagnosis. *Concept:* at least one psychosocial outcome or correlate (e.g., depression, anxiety, mood and emotion regulation, psychological distress, interpersonal functioning, suicidality or self-harm, quality of life, academic or occupational functioning) or a coping mechanism as a primary aim. *Context:* any country or setting. Eligible sources were peer-reviewed primary empirical studies (quantitative, qualitative or mixed methods) published in English between 2010 and 2025. We excluded animal studies; laboratory and biological studies (neuroimaging, hormonal, genetic or physiological aims); intervention and treatment studies; reviews, meta-analyses, guidelines and commentaries; instrument development or validation studies; case reports; books, chapters, dissertations, letters, editorials and conference abstracts; and studies of PMS without a PMDD group.

Selection of Sources of Evidence

All exports were merged into a single library. Duplicates were identified by digital object identifier and by normalised title (Unicode-aware, so that non-Latin titles were matched), and near-identical titles were reviewed manually. Records were then screened in two steps: (a) application of the date, language and publication-type criteria using database metadata, and (b) title, abstract and subject-term screening of every remaining record against the population, concept and study-design criteria. Every decision and exclusion reason was recorded in a screening log (Supplementary File).

[Describe the reviewers: title/abstract screening and data charting were performed with AI assistance (Claude, Anthropic) and must be verified by two independent human reviewers, with disagreements resolved by discussion or a third reviewer. Full-text retrieval and eligibility confirmation are in progress; update the PRISMA-ScR diagram with full-text counts once complete. Declare AI assistance according to the target journal's policy.]

Data Charting

For each included study we charted the author, year, country, study design, sample size, population, method of PMDD ascertainment and the psychosocial domains examined. Country was taken from the abstract or, where not stated, from the study setting described by the authors. Countries were grouped by World Bank region and income group using the fiscal-year 2027 classification (World Bank, 2026); consistent with World Bank usage, LMICs comprise low-, lower-middle- and upper-middle-income economies. Psychosocial outcomes were coded into 13 domains developed iteratively during charting; a study could contribute to more than one domain.

Synthesis

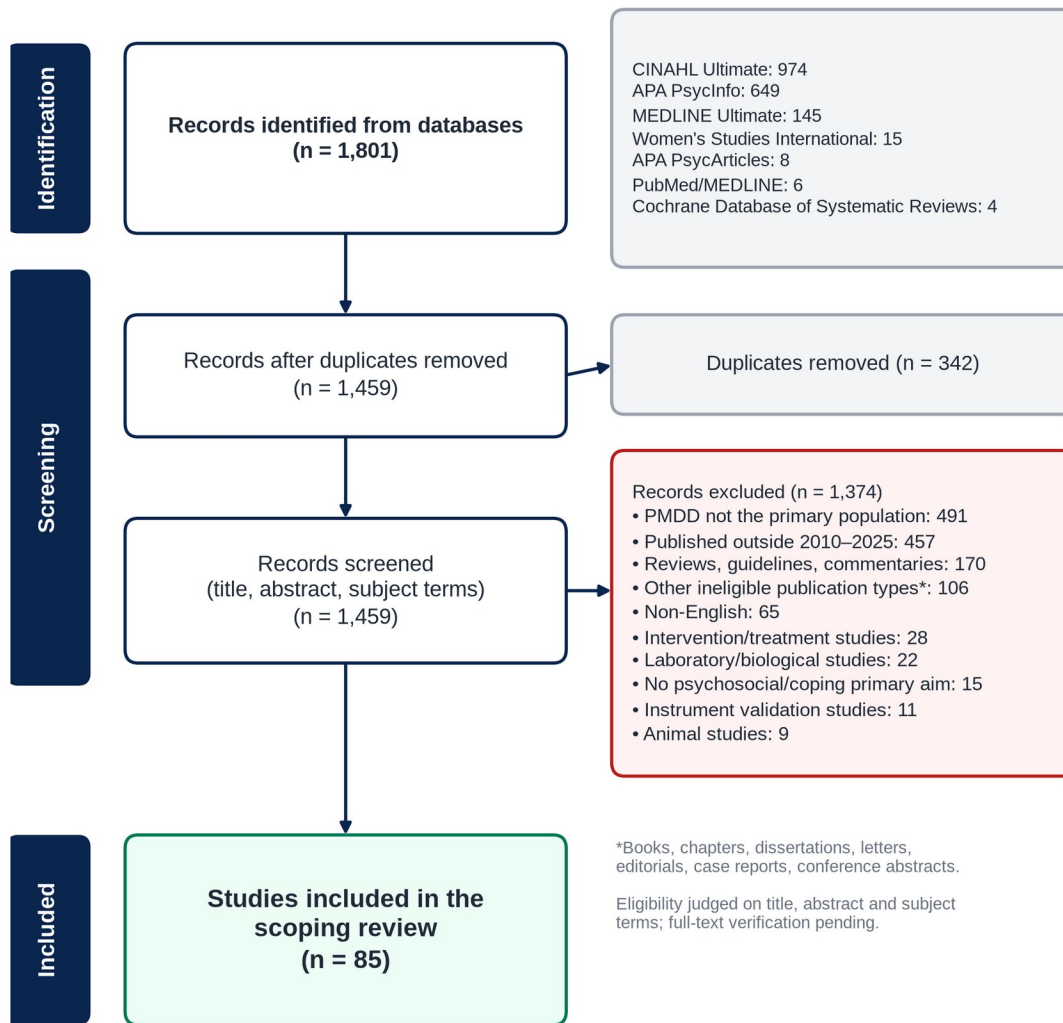
Results were summarised numerically (frequencies and percentages) and narratively by psychosocial domain, with particular attention to the geographic distribution of evidence and to LMIC and sub-Saharan African settings.

Results

Selection of Sources of Evidence

The database exports contained 1,801 records. After 342 duplicates were removed, 1,459 records were screened and 1,374 were excluded: PMDD not the primary study population (n = 491); published outside 2010–2025 (n = 457); reviews, meta-analyses, guidelines, commentaries (n = 170); other ineligible publication types (books, chapters, dissertations, letters, editorials, case reports, conference abstracts) (n = 106); non-english full text (n = 65); intervention or treatment study (n = 28); laboratory/biological study (n = 22); no psychosocial outcome or coping mechanism as a primary aim (n = 15); instrument development/validation study (n = 11); animal study (n = 9). In total, 85 studies met the inclusion criteria (Figure 1).

Figure 1. PRISMA-ScR flow of records



Characteristics of Included Studies

The 85 included studies were published between 2010 and 2025, with almost half (42; 49.4%) published since 2020 and 15 (17.6%) in 2025 alone (Table 1). Most were cross-sectional (48; 56.5%) or case-control (13; 15.3%) designs; 7 used prospective daily ratings or ecological momentary assessment, 6 were cohort or registry studies, 5 were other prospective observational studies, 3 were retrospective record reviews and only 3 were qualitative. Sample sizes ranged from 10 to 1,472,379 participants (median 266). Samples most often comprised community women (n = 17), university or college students (n = 14) and school-age adolescents (n = 7);

others involved population cohorts, psychiatric and gynaecological patients, women with bipolar disorder, attention-deficit/hyperactivity disorder (ADHD) or rheumatoid arthritis, pregnant and perimenopausal women, suicide attempters and online PMDD communities.

Methods of identifying PMDD varied considerably. The largest group of studies (36; 42.4%) relied on retrospective screening instruments or self-report, yielding provisional diagnoses; 20 (23.5%) confirmed PMDD with prospective daily ratings, 13 used structured or psychiatric interviews, 13 used clinical or DSM-criteria diagnoses and 3 used registry diagnoses.

Table 1 *Characteristics of Included Studies (N = 85)*

Characteristic	n	%
§Publication period		
2010–2014	23	27.1%
2015–2019	20	23.5%
2020–2025	42	49.4%
§Study design		
Cross-sectional	48	56.5%
Case-control	13	15.3%
Prospective daily ratings / EMA	7	8.2%
Cohort / registry	6	7.1%
Prospective observational	5	5.9%
Retrospective record review	3	3.5%
Qualitative	3	3.5%
§PMDD ascertainment		
Retrospective screening / self-report (provisional)	36	42.4%
Prospective symptom ratings (confirmed)	20	23.5%
Structured/psychiatric interview	13	15.3%
Clinical/DSM-criteria diagnosis	13	15.3%
Clinical/registry diagnosis	3	3.5%
§World Bank income group (FY2027)		
High income	57	67.1%
Upper-middle income	16	18.8%
Lower-middle income	3	3.5%
Low income	2	2.4%
Not classifiable (online/multinational/not reported)	7	8.2%
§World Bank region		
Europe & Central Asia	29	34.1%
East Asia & Pacific	17	20.0%
North America	15	17.6%
Middle East & North Africa	10	11.8%
Online / multinational / not reported	7	8.2%

Characteristic	n	%
Sub-Saharan Africa	3	3.5%
Latin America & Caribbean	3	3.5%
South Asia	1	1.2%

Note. Percentages are of 85 included studies. Income groups follow the World Bank FY2027 classification

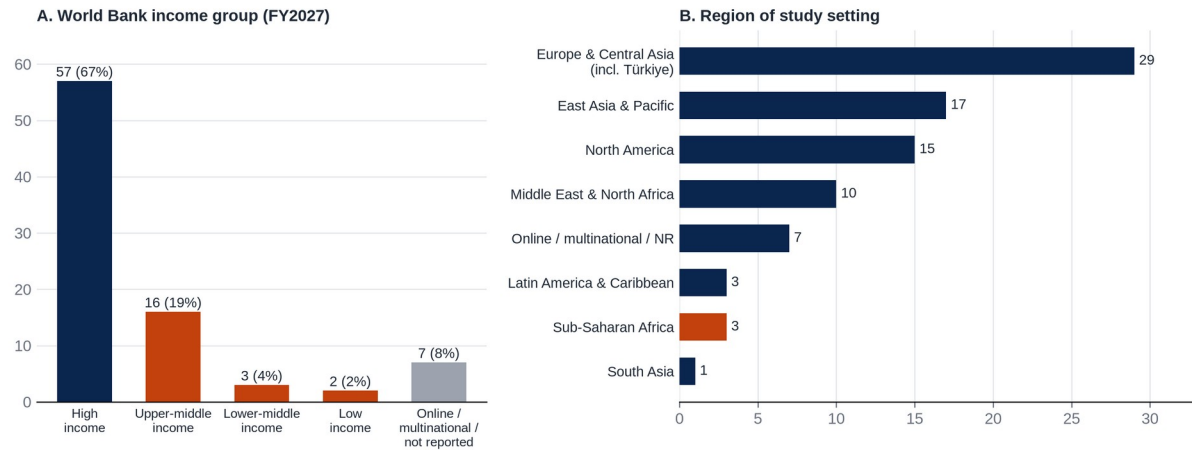
(effective 1 July 2026).

Geographic Distribution

Studies were conducted in 31 countries and territories; a further seven were online, multinational or did not report a country. The United States contributed the most studies (n = 13; 15.3%), followed by Japan, Sweden and Taiwan (n = 6 each), Türkiye (n = 5) and Germany and Iran (n = 4 each). Two-thirds of studies (57; 67.1%) came from high-income economies (Figure 3). Under the World Bank definition, 21 studies (24.7%) were from LMICs, but most of these (16) were from upper-middle-income countries (Türkiye, n = 5; Iran, n = 4; Jordan, Mexico and mainland China, n = 2 each; Brazil, n = 1). Only 5 studies (5.9%) came from lower-middle-income (Bangladesh, Lebanon, Nigeria) or low-income (Ethiopia) countries. Sub-Saharan Africa contributed three studies (3.5%): a survey of 985 secondary-school adolescents in Ibadan, Nigeria (Adegoke et al., 2014), and two studies from Ethiopia—one of 529 university students (Alemu et al., 2017) and one of 548 high-school students (Kibrilew et al., 2024).

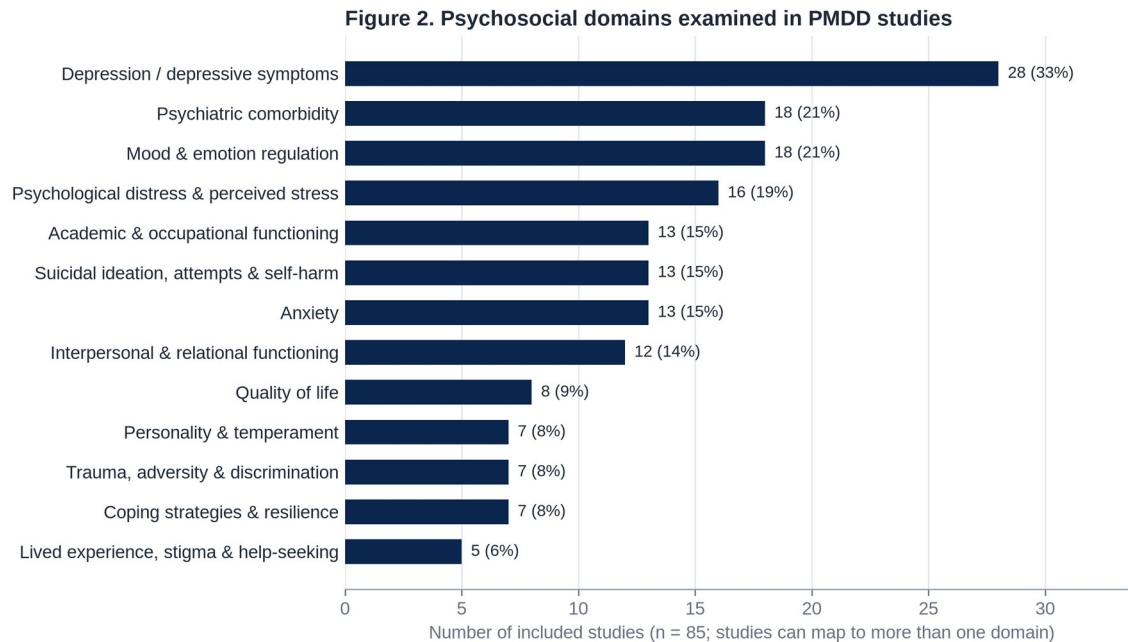
The LMIC evidence also differed in kind. Only one of the 21 LMIC studies (Jordan) confirmed PMDD with prospective daily ratings (Hussein Shehadeh & Hamdan-Mansour, 2018); none of the qualitative or lived-experience studies came from an LMIC; and only one of the seven studies examining coping was conducted in an LMIC (Hamaideh et al., 2014). No study from sub-Saharan Africa examined suicidality, quality of life, interpersonal functioning or coping as a primary aim.

Figure 3. Geographic distribution of included studies (n = 85)



Psychosocial Domains

Figure 2 shows the number of studies examining each psychosocial domain. Depression was the most frequently examined domain (n = 28; 32.9%), followed by mood and emotion regulation and psychiatric comorbidity (n = 18 each), psychological distress and perceived stress (n = 16), anxiety, suicidality and academic or occupational functioning (n = 13 each), interpersonal and relational functioning (n = 12), quality of life (n = 8), personality, trauma and adversity, and coping and resilience (n = 7 each), and lived experience, stigma and help-seeking (n = 5).



Depression and Psychiatric Comorbidity

Women with PMDD reported more depressive symptoms than women with PMS or controls (Balık et al., 2015; Ko et al., 2013; Yen et al., 2018), and PMDD was more common among young women with a history of major depression (Accortt et al., 2013). Longitudinally, a Taiwanese national-insurance cohort of 8,222 women with PMDD and 32,888 matched controls found higher risks of unipolar depression (hazard ratio [HR] 2.58) and bipolar disorder (HR 2.50) after a PMDD diagnosis (Li et al., 2023). PMDD was also associated with antenatal depression (odds ratio [OR] 3.54) (Ihara et al., 2024), perinatal depression (OR up to 3.05) (Schleimann-Jensen et al., 2025) and perimenopausal depression (Flores-Ramos et al., 2010). Comorbidity studies linked PMDD with ADHD—provisional PMDD affected 31.4% of women with a clinical ADHD diagnosis versus 9.8% of a non-ADHD group (Broughton et al., 2025); see also Kondo et al. (2025) and Dorani et al. (2021)—and with bipolar disorder, where comorbid PMDD was associated with earlier onset and greater illness burden (Slyepchenko et al., 2017; Liang et al., 2024; Fornaro & Perugi, 2010). PMDD was associated with bulimia nervosa (OR

7.2) (Nobles et al., 2016) and Internet use disorder (Ko et al., 2014a), and in a Korean national sample 59.3% of women with PMDD had at least one other psychiatric disorder, compared with 21.8% of women without PMDD (Hong et al., 2012).

Mood, Emotion Regulation and Personality

Women with PMDD reported greater difficulties in emotion regulation (Reuveni et al., 2016; Petersen et al., 2016; Torabi et al., 2025; Yen et al., 2018), more rumination and repetitive negative thinking (Beddig et al., 2020; Nayman et al., 2024; Kappen et al., 2022; Craner et al., 2014) and more self-focused attention (Craner et al., 2015; Craner et al., 2016), as well as higher anger and aggression (Akyol et al., 2013; Ducasse et al., 2016; Rezaei & Sajjad, 2019). Ecological momentary assessment studies from Germany showed that favourable habitual emotion-regulation strategies were linked to better everyday mood but did not protect against premenstrual mood deterioration (Nayman et al., 2023), whereas momentary rumination and present-moment awareness predicted late-luteal mood (Nayman et al., 2024). Personality studies consistently reported higher neuroticism and related traits (Izadi-Mazidi & Amiri, 2019; Gingnell et al., 2010; Fernández et al., 2019; Miller et al., 2010), as well as anxious and cyclothymic temperaments (Ekmekçi Ertek et al., 2023) and differences in reinforcement sensitivity (Ko et al., 2014b).

Psychological Distress and Anxiety

PMDD was associated with psychological distress in a Swiss population survey (Tschudin et al., 2010) and PMDD or premenstrual symptom severity was associated with higher perceived stress in Jordan, Spain, Saudi Arabia and Ethiopia (Hamaideh et al., 2014; Fernández et al., 2019; Innab et al., 2025; Kibrilew et al., 2024); women with PMDD in Germany reported particularly high daily-life stress in the late luteal phase (Beddig et al., 2019); in a Spanish case-

control study, high perceived stress was associated with more than five-fold higher odds of PMDD (OR 5.79) (Fernández et al., 2019). In Ethiopian high-school students, depressive symptoms and high perceived stress were among the factors associated with PMDD (Kibralew et al., 2024). Anxiety was elevated in several samples (Lee et al., 2024; Kappen et al., 2022; Roy et al., 2025), including higher health anxiety and anxiety sensitivity among college women with provisional PMDD (Craner et al., 2013).

Suicidality and Self-Harm

Thirteen studies examined suicidality. In a nationally representative US sample, PMDD was associated with suicidal ideation (OR 2.22), plans (OR 2.27) and attempts (OR 2.10) after adjustment for psychiatric comorbidity (Pilver et al., 2013). Among Bangladeshi university students, PMDD was associated with suicidal ideation (adjusted OR [AOR] 5.42) and attempts (AOR 4.07) (Roy et al., 2025). In a global survey of 599 people with prospectively diagnosed PMDD, 72% reported lifetime active suicidal ideation, 34% a suicide attempt and 51% non-suicidal self-injury (Eisenlohr-Moul et al., 2022), and 39.1% of Swedish women with confirmed PMDD reported current suicidal ideation in the late luteal phase (Wikman et al., 2022). Among women with mood disorders in Taiwan, PMDD was independently associated with lifetime suicide attempts (OR 3.46) (Chan et al., 2021); PMDD was more frequent among Iranian suicide attempters than matched controls (Shams-Alizadeh et al., 2018); and 23% of women hospitalised after a suicide attempt in France met PMDD criteria (Ducasse et al., 2016); and a Swedish population cohort found a higher risk of suicidal behaviour (HR 2.26) among women with clinically recognised premenstrual disorders (Yang et al., 2021).

Functioning, Relationships and Quality of Life

PMDD was associated with presenteeism and absenteeism in a nationwide Japanese workforce survey (Maeda et al., 2025), with absenteeism from clinical training among Kuwaiti nursing students (Omu et al., 2011), with differences in academic self-determination among Jordanian university students (Hussein Shehadeh & Hamdan-Mansour, 2018) and with lower occupational competence in Türkiye (Pekçetin et al., 2022); in contrast, an Ethiopian study found no significant association between premenstrual symptoms and academic performance (Alemu et al., 2017). PMDD was associated with relationship disruption among married or cohabiting women in a Swedish cohort (incidence rate ratio 1.22) (Westermarck et al., 2024), and both people with PMDD and their partners reported lower relationship quality (Hodgetts & Kinghorn, 2025). Quality of life was consistently lower: in a Swedish cohort of 17,284 women, the association with reduced quality of life was stronger for PMDD than for PMS (Wang et al., 2025); untreated Japanese patients had a mean EQ-5D score of 0.795, corresponding to an estimated lifetime loss of about three quality-adjusted life years (Yamada & Kamagata, 2017); and Iranian adolescents with PMDD scored lower on all SF-36 domains except physical functioning (Delara et al., 2012).

Trauma, Adversity and Discrimination

Adverse childhood experiences were associated with premenstrual disorders in a dose-dependent manner in Iceland (prevalence ratio 2.46 for four or more experiences), more strongly for PMDD than PMS (Yang et al., 2022), and four or more such experiences were associated with PMDD among Japanese working women (AOR 5.61) (Kanamori et al., 2025); school bullying was associated with premenstrual disorders among Chinese adolescents, more strongly for PMDD than PMS (Liao et al., 2025); 83% of Australian clinic patients with PMDD reported

early-life trauma (Kulkarni et al., 2022); and perceived discrimination was associated with PMDD among US minority women (Pilver et al., 2011). In Lebanon, depression mediated the association between childhood abuse and PMDD (Younes et al., 2021).

Coping and Lived Experience

Only seven studies examined coping or resilience. Jordanian women with PMS or PMDD most often self-treated by taking analgesics, drinking hot fluids, wearing warm clothing and lying on the abdomen (Hamaideh et al., 2014); Canadian women with PMDD used cannabis more pre-menstrually and menstrually, partly for coping motives (Joyce et al., 2021); women with premenstrual disorders in the United States reported more maladaptive coping styles than controls (Craner et al., 2015); and in a Spanish case-control study, higher use of most coping strategies was associated with increased odds of PMDD (Fernández et al., 2019). Coping styles were also measured alongside rumination in a US study (Craner et al., 2014). Resilience potentially moderated associations between high-risk PMDD and anxiety, depression and loneliness among Hong Kong students (Lee et al., 2024), and members of an online PMDD community described peer support and knowledge as sources of agency (Winslow et al., 2023). Five studies, all from high-income or online settings, explored lived experience and help-seeking, describing misdiagnosis and “lost decades” (Osborn et al., 2020), medical mistrust and masking (Habib et al., 2025), variable provider knowledge across specialties (Hantsoo et al., 2022) and patient priorities for PMDD-tailored mental health care, including suicidality and relationships (Chenji et al., 2025).

Discussion

This scoping review mapped 85 empirical studies of psychosocial outcomes and coping among women with PMDD. Three findings stand out. First, the literature is growing quickly—

half of the studies were published since 2020—and it consistently links PMDD with depression, psychiatric comorbidity, suicidality, impaired functioning and reduced quality of life. The suicidality findings are particularly striking and span nationally representative surveys, clinical samples and population registries, supporting calls for routine suicide-risk screening in people with PMDD. Second, the evidence is narrow in scope: coping ($n = 7$) and lived experience ($n = 5$) were rarely primary aims, and no study examined adaptive coping longitudinally. Third, the evidence base is geographically concentrated: two-thirds of studies came from high-income countries, only five came from low- or lower-middle-income countries, and just three came from sub-Saharan Africa.

Methodological features limit what the evidence can tell us. Fewer than one in four studies confirmed PMDD with prospective daily ratings, and over two in five relied on provisional, retrospective diagnoses. Provisional diagnoses substantially overestimate prevalence (Reilly et al., 2024), and in one study self-reported PMDD (51.8%) far exceeded clinically assessed PMDD (5.9%) (Senín-Calderón et al., 2023). Associations with psychosocial outcomes based on provisional diagnoses may therefore conflate PMDD with PMS or with premenstrual exacerbation of other disorders. The problem is sharpest in LMICs, where only one study used prospective ratings; the high prevalence (33.0%) reported among Ethiopian high-school students from a single cross-sectional DSM-5-based assessment (Kibrilew et al., 2024) illustrates the need for validated, prospectively confirmed assessment. Notably, none of the 11 instrument-validation studies identified (and excluded) during screening was conducted in an African setting.

The scarcity of evidence from sub-Saharan Africa is not evidence of a small burden. Premenstrual symptoms are common across Africa (Andualem et al., 2024), and a community-

based study of adolescent girls in southwest Ethiopia—excluded here because PMDD was a covariate rather than the study population—found PMDD independently associated with suicidal ideation (Segon et al., 2025). Research on premenstrual coping in Ethiopia has so far focused on PMS rather than PMDD (Eshetu et al., 2022). Together these observations suggest that PMDD’s psychosocial burden in the region is real but largely invisible to health systems.

Implications for Public Health Practice and Research

For practice, the consistent association between PMDD and suicidality supports routine screening for suicidal ideation in people presenting with premenstrual mood symptoms, and vice versa. PMDD screening could be integrated into existing sexual and reproductive health, school-health and university health services, which reach the adolescents and students who dominate the LMIC evidence. Provider training should address the diagnostic delays and dismissive care that patients describe. For research, priorities include prospectively confirmed studies in LMICs; qualitative research on lived experience, stigma and help-seeking in African contexts; validation of PMDD instruments in African languages; and longitudinal studies of coping and of interventions that strengthen adaptive coping.

Strengths and Limitations

This review screened records from seven databases, documented every decision in an auditable log and applied the World Bank’s current income classification. Several limitations apply. First, eligibility was judged on titles, abstracts and subject terms; full-text review is ongoing and may change the final set of studies. Second, screening and charting were AI-assisted and require independent verification by two human reviewers. Third, restricting to English excluded 65 records (most commonly Persian, French, Korean, German and Turkish), and the databases searched did not include regional sources such as African Journals Online, Embase,

Scopus or grey literature, so the geographic gap may be partly an artefact of database coverage. Fourth, some studies combined PMS and PMDD, country was occasionally inferred from the study setting, and domains were coded from abstracts, so domain counts are approximate.

Conclusions

Evidence on the psychosocial burden of PMDD is growing and points to substantial effects on mental health, suicidality, functioning, relationships and quality of life. It remains concentrated in high-income countries, relies heavily on provisional diagnoses and rarely examines coping or lived experience. Culturally responsive screening, integration of PMDD into reproductive and mental health services, and Africa-centred research investment are needed for a condition whose burden remains largely invisible to health systems globally.

Declarations

[Funding: state funding or “none”. Conflicts of interest: declare. Author contributions (CRediT): complete. AI use: describe AI assistance with screening, charting and drafting per journal policy. Data availability: screening log and charting table provided as Supplementary File.]

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Supplementary Table S1

Characteristics of the 85 Included Studies

Study	Country	Design	N	Population	PMDD ascertainment	Psychosocial domains
Tschudin et al. (2010)	Switzerland	Cross-sectional (national health survey)	3913	Population-based sample	Screening tool (PSST-based)	Distress/stress
Takeda et al. (2010)	Japan	Cross-sectional	618	High school students	Screening questionnaire	Academic/occupational functioning
Flores-Ramos et al. (2010)	Mexico	Cross-sectional	141	Perimenopausal clinic patients	History of PMDD (clinical assessment)	Depression
Yang et al. (2010)	United States	Cross-sectional (online survey, secondary analysis)	949	Online community sample	Screening tool (PMSIS)	Quality of life; Interpersonal
Gingnell et al. (2010)	Sweden	Case-control	85	Clinical patients and controls	Prospective confirmation	Personality
Fornaro & Perugi (2010)	Italy	Cross-sectional	92	Women with bipolar disorder	Structured interview (SCID-I)	Psychiatric comorbidity
Heinemann et al. (2010)	Multinational	Prospective web-based study	822	Community women	Prospective (DRSP)	Quality of life; Interpersonal; Academic/occupational functioning
Miller et al. (2010)	United States	Case-control	124	Clinical research participants	Prospective confirmation	Personality
Nowosielski et al. (2010)	Poland	Cross-sectional (population study)	1540	Community women	Questionnaire-based diagnosis	Interpersonal
Pilver et al. (2011)	United States	Cross-sectional (national surveys)	2718	Asian, Latina and Black women	Diagnostic interview (WMH-CIDI)	Trauma/adversity
Omu et al. (2011)	Kuwait	Prospective observational	110	Nursing students	Prospective (Arabic DRSP)	Academic/occupational functioning
Hong et al. (2012)	South Korea	Cross-sectional (nationwide survey)	2499	Nationally representative sample	Diagnostic interview (K-CIDI)	Psychiatric comorbidity; Suicidality/self-harm; Depression
Delara et al. (2012)	Iran	Cross-sectional	602	School adolescents	DSM-IV / ICD-10 criteria	Quality of life; Interpersonal
Accortt et al. (2013)	United States	Case-control	164	Young women with/without depression	Structured interview (SCID-PMDD)	Depression; Psychiatric comorbidity
Ko et al. (2013)	Taiwan	Case-control	142	Community women	Psychiatric interview + 3-cycle follow-up	Depression; Anxiety; Mood/emotion regulation; Academic/occupational functioning
Pilver et al. (2013)	United States	Cross-sectional (national survey, CPES)	3965	Nationally representative sample	Diagnostic interview (DSM-IV)	Suicidality/self-harm
Craner et al. (2013)	United States	Cross-sectional	731	College women	Screening tool (provisional diagnosis)	Anxiety; Quality of life
Akyol et al. (2013)	Türkiye	Case-control	100	Clinical patients and controls	DSM criteria	Mood/emotion regulation
Hamaideh et al. (2014)	Jordan	Cross-sectional	254	University students/staff	DSM-IV-TR research criteria	Distress/stress; Coping/resilience; Academic/occupational functioning
Ko et al. (2014b)	Taiwan	Case-control	142	Community women	Psychiatric interview +	Personality; Depression;

Study	Country	Design	N	Population	PMDD ascertainment	Psychosocial domains
					2-cycle prospective	Anxiety
Craner et al. (2014)	United States	Cross-sectional + prospective (two studies)	735 / 85	College women	Retrospective screening + 60-day prospective	Mood/emotion regulation; Coping/resilience; Anxiety
Ko et al. (2014a)	Taiwan	Case-control	155	Community women	Psychiatric interview	Psychiatric comorbidity; Distress/stress; Mood/emotion regulation
Adegoke et al. (2014)	Nigeria	Cross-sectional	985	Secondary school adolescents	Self-report questionnaire	Distress/stress
Balık et al. (2015)	Türkiye	Cross-sectional	92	Gynaecology outpatients	Clinical diagnosis	Depression; Anxiety; Quality of life; Academic/occupational functioning
Craner et al. (2015)	United States	Case-control (laboratory stressor)	52	Community women	Clinical interview + 30-day prospective ratings	Mood/emotion regulation; Coping/resilience
Petersen et al. (2016)	United States	Case-control	36	Community women	Prospective daily ratings (DSM-5)	Mood/emotion regulation; Interpersonal; Distress/stress
Craner et al. (2016)	United States	Prospective daily ratings (30 days)	61	Community women	Prospective daily ratings	Mood/emotion regulation
Nobles et al. (2016)	United States	Cross-sectional (national survey, CPES)	8694	Nationally representative sample	Diagnostic interview (DSM-IV)	Psychiatric comorbidity
Ducasse et al. (2016)	France	Cross-sectional	232	Hospitalised suicide attempters	Algorithm from shortened PAF	Personality; Suicidality/self-harm; Mood/emotion regulation
Reuveni et al. (2016)	Israel	Cross-sectional	648	University students	Screening tool (PSST)	Mood/emotion regulation
Schmalenberger et al. (2017)	United States	Prospective daily ratings	267	Community women	Prospective (DRSP)	Academic/occupational functioning; Interpersonal
Alemu et al. (2017)	Ethiopia	Cross-sectional	529	University students	DSM-based criteria	Academic/occupational functioning
Yamada & Kamagata (2017)	Japan	Chart review	66	Untreated clinical patients	Clinical diagnosis	Quality of life
Slyepchenko et al. (2017)	United States	Secondary analysis (STEP-BD cohort)	1099	Women with bipolar disorder	DSM-5 criteria (self-report questionnaire)	Psychiatric comorbidity; Depression
Shams-Alizadeh et al. (2018)	Iran	Case-control	240	Hospitalised suicide attempters and controls	Psychiatric interview (DSM-5)	Suicidality/self-harm
Hussein Shehadeh & Hamdan-Mansour (2018)	Jordan	Prospective-correlational	858	University students	Daily symptom records	Academic/occupational functioning
Yen et al. (2018)	Taiwan	Case-control	196	Community women	Psychiatric interview + 3-cycle prospective	Mood/emotion regulation; Depression; Anxiety; Distress/stress
de Carvalho et al. (2018)	Brazil	Cross-sectional	727	Young adult community	Diagnostic interview	Psychiatric comorbidity;

Study	Country	Design	N	Population	PMDD ascertainment	Psychosocial domains
		(population-based)		women	(MINI-Plus)	Suicidality/self-harm; Depression; Anxiety
Izadi-Mazidi & Amiri (2019)	Iran	Cross-sectional	210	University students	Screening tool (PSST)	Personality
Fernández et al. (2019)	Spain	Case-control	264	Hospital and family-planning clinic attendees	Clinical diagnosis	Distress/stress; Personality; Coping/resilience
Beddig et al. (2019)	Germany	Ambulatory assessment (EMA)	122	Community women	Prospective daily ratings	Distress/stress; Mood/emotion regulation
Śliwerski & Bielawska-Batorowicz (2019)	Poland	Experimental cross-sectional	293	Community women	Screening tool (PSST)	Mood/emotion regulation; Depression
Rezaei & Sajjad (2019)	Iran	Cross-sectional	475	High school students	Screening tool (PSST)	Mood/emotion regulation; Depression
Beddig et al. (2020)	Germany	Ambulatory assessment (EMA)	122	Community women	Prospective daily ratings	Mood/emotion regulation
Osborn et al. (2020)	United Kingdom	Qualitative (interviews)	17	Clinically diagnosed women	Specialist diagnosis + PSST	Lived experience/help-seeking; Distress/stress
Joyce et al. (2021)	Canada	Daily diary (32 days)	69	Female cannabis users	Clinical interview + prospective mood	Depression; Coping/resilience; Psychiatric comorbidity
Dorani et al. (2021)	Netherlands	Cross-sectional	209	Adult ADHD outpatients	Diagnostic interview (MINI-Plus)	Psychiatric comorbidity
Yang et al. (2021)	Sweden	Population registry cohort + sibling analysis	1472379	National population	Clinical indication (registry)	Suicidality/self-harm
Younes et al. (2021)	Lebanon	Cross-sectional (online survey)	NR	University students	Self-report PMDD measure	Trauma/adversity; Depression; Distress/stress
Chan et al. (2021)	Taiwan	Cross-sectional	383	Outpatients with mood disorders	Diagnostic interview (SADS-L)	Suicidality/self-harm; Psychiatric comorbidity
Kappen et al. (2022)	Belgium	Cross-sectional + two-phase follow-up	380	Community women	Screening tool (PSST)	Depression; Anxiety; Distress/stress; Mood/emotion regulation
Pekçetin et al. (2022)	Türkiye	Cross-sectional	70	University students	DSM-5 criteria	Academic/occupational functioning
Eisenlohr-Moul et al. (2022)	Online (international)	Cross-sectional (global online survey)	599	People with prospectively diagnosed PMDD	Self-reported prospective diagnosis	Suicidality/self-harm; Psychiatric comorbidity
Wikman et al. (2022)	Sweden	Cross-sectional (trial baseline)	110	Clinically confirmed PMDD	Prospective daily ratings (DSM-5)	Suicidality/self-harm; Depression
Kulkarni et al. (2022)	Australia	Retrospective clinical database	100	Specialist clinic patients	Clinical diagnosis	Trauma/adversity
Groenman et al. (2022)	Netherlands	Cross-sectional	70	Autistic and non-autistic women	Diagnostic assessment	Psychiatric comorbidity
Yang et al. (2022)	Iceland	Cross-sectional (SAGA cohort)	11973	Population-based cohort	Modified PSST	Trauma/adversity
Hantsoo et al. (2022)	Online (international)	Cross-sectional (IAPMD)	2512	Patients seeking care for	Self-reported	Lived experience/help-

Study	Country	Design	N	Population	PMDD ascertainment	Psychosocial domains
		global survey)		PMDD	symptoms/diagnosis	seeking
Ekmekçi Ertek et al. (2023)	Türkiye	Cross-sectional (online survey)	408	Healthy women	Screening tool (PSST)	Personality; Suicidality/self-harm; Psychiatric comorbidity
Winslow et al. (2023)	Online (international)	Qualitative (thematic analysis of forum posts)	NR (top posts)	Online PMDD community (r/PMDD)	Self-identified	Coping/resilience; Lived experience/help-seeking; Interpersonal
Senín-Calderón et al. (2023)	Spain	Cross-sectional	105	General population and MDD patients	Semi-structured interview (DSM-5)	Depression; Academic/occupational functioning; Interpersonal
Li et al. (2023)	Taiwan	Nationwide registry cohort	41110	National insurance database	Clinical diagnosis (registry)	Depression; Psychiatric comorbidity
Akyuz Cim & Cim (2023)	Türkiye	Case-control	62	Psychiatric outpatients	Clinical diagnosis	Depression
Nayman et al. (2023)	Germany	Ambulatory assessment (EMA)	122	Community women	Prospective daily ratings	Mood/emotion regulation; Depression
Ihara et al. (2024)	Japan	Cross-sectional	212	Pregnant women	PMDD scale	Depression
Lee et al. (2024)	China (Hong Kong)	Cross-sectional	541	University students	Screening tool (high-risk PMDD)	Depression; Anxiety; Interpersonal; Coping/resilience
Nayman et al. (2024)	Germany	Ambulatory assessment (EMA)	120	Community women	Prospective daily ratings	Mood/emotion regulation
Westermarck et al. (2024)	Sweden	Prospective cohort (registry-linked)	15606	Population-based cohort	Modified PSST	Interpersonal
Kibralew et al. (2024)	Ethiopia	Cross-sectional	548	High school students	DSM-5 criteria	Depression; Distress/stress
Liang et al. (2024)	China	Cross-sectional	262	Women with bipolar disorder	Screening tool (PSST)	Psychiatric comorbidity; Depression
Schleimann-Jensen et al. (2025)	Sweden	Longitudinal cohorts (two)	NR	Pregnant/postpartum women	Retrospective self-report	Depression; Anxiety
Hodgetts & Kinghorn (2025)	United Kingdom	Cross-sectional (online survey)	403 / 151	People with PMDD, partners, controls	Self-reported diagnosis	Quality of life; Interpersonal
Broughton et al. (2025)	NR (online panel)	Cross-sectional (online survey)	715	Young adults (18-34)	Screening tool (PSST)	Psychiatric comorbidity; Depression; Anxiety
Torabi et al. (2025)	NR	Cross-sectional (phase comparison)	150	University students	Clinical interview + screening tool	Anxiety; Mood/emotion regulation; Distress/stress
Antosz-Rekucka & Prochwicz (2025)	Poland	Cross-sectional + prospective	228 / 90	Community women	Retrospective + prospective diagnosis	Trauma/adversity; Distress/stress; Depression
Innab et al. (2025)	Saudi Arabia	Cross-sectional	144	Nursing students	Screening tool (PSST)	Distress/stress
Kondo et al. (2025)	Japan	Retrospective chart review	290	Adolescent clinical patients	PMDD assessment scale	Psychiatric comorbidity; Depression
Wang et al. (2025)	Sweden	Cross-sectional + prospective (LifeGene cohort)	17284	Population-based cohort	Modified PSST + registry diagnoses	Quality of life
Habib et al. (2025)	Canada	Qualitative (interviews)	10	People living with	Self-identified/diagnosed	Lived experience/help-

Study	Country	Design	N	Population	PMDD ascertainment	Psychosocial domains
				PMDD		seeking; Suicidality/self-harm
Chenji et al. (2025)	Online (international)	Cross-sectional (online survey)	308	PMDD patients (IAPMD followers)	Self-reported diagnosis	Lived experience/help-seeking; Suicidality/self-harm; Interpersonal
Maeda et al. (2025)	Japan	Cross-sectional (nationwide online survey)	4818	General workforce	Self-report screening	Academic/occupational functioning
Bardan-Inchaustegui et al. (2025)	Mexico	Cross-sectional	134	Women with rheumatoid arthritis and controls	Screening tool (PSST)	Academic/occupational functioning
Liao et al. (2025)	China	Cross-sectional	10042	School adolescents	Modified C-PEX questionnaire (screening)	Trauma/adversity; Psychiatric comorbidity
Roy et al. (2025)	Bangladesh	Cross-sectional	516	University students	Screening tool (PSST)	Suicidality/self-harm; Depression; Anxiety; Distress/stress
Kanamori et al. (2025)	Japan	Cross-sectional (online survey)	2000	Full-time working women	PMDD scale	Trauma/adversity

Note. NR = not reported in the abstract. Country was taken from the abstract or study setting. Full screening decisions for all 1,801 records are in the Supplementary File (Excel).